

**Laboratory Investigation Report**

Patient Name	: Seema Sharma	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 24 Y 1 M 17 D /F	OP/IP No	: OP/97842
Max ID/Mobile	: 411360/8752005200	Collection Date/Time	: 18/Aug/2021 02:50PM
Lab ID	: 1266082100427	Receiving Date	: 19/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 19/Aug/2021

Blood Bank**Blood Grouping and RH Factor*, EDTA**

Date	18/Aug/2021 02:50PM	Unit	Bio Ref Interval
Blood Group Haemagglutination	B POSITIVE		

Kindly correlate with clinical findings

***** End Of Report *****Dr. Sangeeta Pathak , DIHBT
Head-Transfusion Med

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SIN No:B2B586871, Test Performed at :1103 - Max Hospital Saket(East Block), 1, 2, Press Enclave Marg, Saket Institutional Area, Saket, New Delhi

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8752 1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

The authenticity of the report can be verified by scanning the Q R Code on top of the page

CIN No.: L72200MH2001PLC322854

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Clinical Biochemistry

KFT Profile with Calcium,Uric Acid*, Serum

Date	18/Aug/2021 02:50PM	Unit	Bio Ref Interval
Urea Enzymatic Rate (Urease)	19.6	mg/dL	17.12 - 55.64
Creatinine Rate-Jaffe	0.60	mg/dl	0.61 - 1.24
eGFR MDRD	122.69	ml/min/1.73 m ²	
Uric Acid Enzymatic Trinder	5.20	mg/dl	2.6 - 8.0
Calcium (Total) ISE Indirect	9.45	mg/dl	8.9 - 10.3
Sodium ISE Indirect	140.1	mmol/L	136 - 144
Potassium ISE Indirect	4.50	mmol/L	3.6 - 5.1
Chloride ISE Indirect	104.1	mmol/l	101-111
Bicarbonate ISE Indirect	24.8	mmol/l	22-32

Interpretation Ref. Range

eGFR - Estimated Glomerular Filtration Rate is calculated by MDRD equation which is most accurate for GFRs ≤ 60 ml / min / 1.73 m². MDRD equation is used for adult population only.

<60ml / min / 1.73 m² - Chronic Kidney Disease

<15 ml / min / 1.73 m² - Kidney failure



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Clinical Biochemistry

Liver Function Test Profile, Serum*

Date	18/Aug/2021 02:48PM	Unit	Bio Ref Interval
Total Protein Biuret	7.32	g/dl	6.5 - 8.1
Albumin BCP	5.0	g/dl	3.5 - 5.0
Globulin Calculated	2.4	g/dl	2.3 - 3.5
A.G. ratio Calculated	2.1		1.2 - 1.5
Bilirubin (Total) Diazo	0.32	mg/dl	0.3 - 1.2
Bilirubin (Direct) Diazo	0.12	mg/dl	0.1 - 0.5
Bilirubin (Indirect) Calculated	0.20	mg/dL	0.1 - 1.0
SGOT- Aspartate Transaminase (AST) UV without P5P	26.13	U/L	10 -35
SGPT- Alanine Transaminase (ALT) Kinetic Rate using LDH	20.97	U/L	< 40
Alkaline Phosphatase PNP AMP Buffer	110.55	U/L	32 - 91
GGTP (Gamma GT), Serum Enzymatic Rate	15.7	U/L	7 - 50



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Clinical Biochemistry**Blood Sugar (Random), Fluoride Plasma***

Date	18/Aug/2021 02:50PM	Unit	Bio Ref Interval
Random Glucose UV-Hexokinase	78.43	mg/dl	74 - 140

Interpretation A random plasma glucose of > 200 mg/dl with symptoms of diabetes is diagnostic. A confirmatory fasting plasma Glucose (FPG) test or OGTT should be completed on a different day if the clinical condition of the patient permits.

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Hematology

APTT (Partial Thromboplastin Time), Citrate Plasma*, Sodium Citrate

Date	18/Aug/2021 02:48PM	Unit	Bio Ref Interval
APTT	30.6	Sec	24.9 - 33.3
Photo-Optical-Nephelometry			
Control	28.0	sec	

Interpretation

(Activated Partial Thromboplastin Time; PTT)

APTT is the test which checks the "intrinsic coagulation" pathway and is useful for detecting screening of haemophilia A & B, screening of coagulation inhibitors like lupus anticoagulant.

APTT can also be used to monitor heparin monitoring.

Raised APTT is found in - Liver disease, DIC, heparin treatment, circulating inhibitors (Lupus Anticoagulant), and haemophilia (deficiency of Factor VIII & IX).

Low APTT may be seen in - high concentration Factor VIII and is independent risk factor for increased incidence of thrombus formation.

Advice: - 'APTT mixing study', 'Lupus Anticoagulant test', 'specific Factor(s) assay' may be added on for further evaluation.

Prothrombin Time (with INR), Citrate Plasma*

Date	18/Aug/2021 02:48PM	Unit	Bio Ref Interval
Prothrombin Time (PT)	12.8	Sec	12.1 - 15.1
Photo-Optical-Nephelometry			
MNPT Value	11.4	Sec	
INR	1.13		

Interpretation

(Syn: - Prothrombin Time)

PT is the test which checks the "extrinsic coagulation" pathway and is useful for detecting coagulation deficiency, liver disease and disseminated intravascular Coagulation (DIC).

PT can also be expressed as International normalized ratio (INR) used for monitoring warfarin therapy.

Raised PT value seen in - factor deficiency (Fibrinogen (I), Prothrombin (II), factor V, VII, X), oral anticoagulation therapy, liver diseases, Vitamin K deficiency and DIC.

Advice: - 'PT mixing study', 'specific factor(s) assay' may be added on for further evaluation.



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Hematology

Kindly correlate with clinical findings

*** End Of Report ***


Dr. Mrinalini Priyadarshini
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Immunoassay

TSH,Serum*

Date	18/Aug/2021	Unit	Bio Ref
	02:50PM		Interval
Thyroid Stimulating Hormone	1.9	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

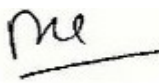
Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Poonam. S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services



Dr. Dilip Kumar M.D.
Associate Director &
Manager Quality



Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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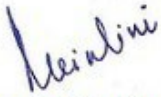
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Immunoassay

Dr. Mrinalini Priyadarshini
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Serology Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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Rapid Card Test - HCV*, Serum

Immunochromatography

HCV Card Test	Negative
---------------	----------

Comment Interpretation

It is only a screening test. All reactive samples should be confirmed by HCV quantitative PCR
A non reactive result does not exclude the possibility of exposure to or infection with HCV.
Patients with auto immune liver disease may show falsely reactive results

Rapid Card Test - HIV I & II*, Serum

Immunochromatography

Rapid Card HIV I & II	Negative
-----------------------	----------

Comment This is only a screening test. All samples detected reactive must be confirmed by using HIV RNA PCR / three different methods.
A non- reactive test result does not exclude the possibility of exposure to an infection with HIV (Window Period) or low viral load.

Advice:

1. HIV by 3 different methods
2. Confirmatory HIV RNA Quantitative PCR.



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Serology Viral Markers (Rapid)

Test Name	Result	Unit	Bio Ref Interval
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VDRL/RPR*, Serum

RPR(Syphilis) Slide Flocculation	Non Reactive
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Comment

Interpretation

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Rapid Card Test - Hepatitis B Surface Antigen*, Serum

Immunochromatography

Rapid Card HBsAg	Negative
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Comment

Interpretation

This is only a screening test.

All reactive samples should be confirmed by 'HBsAg confirmatory test or HBV DNA PCR'.

A non – reactive does not exclude the possibility of exposure to infection with HBV (window period)

False positive results can be obtained due to the presence of other antigens or elevated levels of RF factor.

Advise: Confirmatory test 'HBsAg Confirmatory Quantitative' test followed by 'HBV DNA quantitative PCR'

Kindly correlate with clinical findings

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**Serology
Viral Markers (Rapid)**

Test Name	Result	Unit	Bio Ref Interval
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Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 19/Aug/2021

Test Name	Immunoassay	Result	Unit	Bio Ref Interval
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Progesterone, Serum

Progesterone
CLIA

0.83

ng/mL

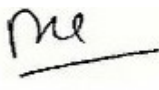
Ref. Range

Males :	0.1 - 0.84
Non Pregnant Females:	
Mid Follicular Phase :	0.31 - 1.52
Mid Luteal Phase :	5.16 - 18.56
Post Menopausal:	0.08 - 0.78
Pregnant Females:	
First Trimester :	4.73 - 50.74
Second Trimester :	19.41 - 45.30

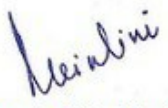
Kindly correlate with clinical findings

*** End Of Report ***


Dr. Poonam S. Das, M.D.
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Associate Director &
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SIN No: B2B586871, Test Performed at : 910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017

Booking Centre : 1534 - Arctern Healthcare Noida, 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002,
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CIN No.: L72200MH2001PLC322854

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Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

